

Testosterone Replacement Therapy

Men vs Women — delivery methods, dosing, monitoring, and managing side effects

HORMONE REFERENCE

Danielle Alcala · nopriorauthorization.com

TRT DELIVERY METHODS – COMPARISON

Method	Frequency	Pros	Cons	Best For
Subcutaneous injection (testosterone cypionate)	Weekly or twice weekly	Stable levels, cost-effective, precise dosing	Requires self-injection, injection site reactions	Men wanting precise dose control and lowest cost
Intramuscular injection	Every 1–2 weeks	Effective, inexpensive	Level peaks and troughs more pronounced	Men who prefer less frequent injection
Topical gel / cream	Daily	Non-invasive, easy to adjust	Transfer risk to partners/children, skin reactions, inconsistent absorption	Men or women with needle aversion
Transdermal patch	Daily	Consistent delivery, non-invasive	Skin irritation, patch falls off, less flexible dosing	Men preferring patch convenience
Pellet implant	Every 4–6 months (M) / 3–4 months (F)	No daily compliance needed, stable levels	Irreversible until dissolved, procedure required, cannot adjust	Patients wanting maximum convenience
Buccal (Striant)	Twice daily	Avoids injection, no transfer risk	Gum irritation, compliance challenge, not widely used	Limited use cases

TRT IN WOMEN – DOSING REFERENCE

Delivery Method	Starting Dose	Maintenance Range	Target Level
Testosterone cream / gel (compounded)	0.5–1mg daily	1–5mg daily	Total T 50–80 ng/dL, Free T 1–3 pg/mL
Subcutaneous injection (compounded)	2–5mg weekly	2–10mg weekly	Same targets. Less common in women
Pellet (female)	50–100mg per insertion	75–200mg per insertion	Total T 80–150 ng/dL at 4–6 wk draw
Note	Women are FAR more sensitive to testosterone than men	—	Start low, go slow. Androgenic side effects develop at much lower doses than men

TRT IN MEN – DOSING REFERENCE

Delivery Method	Starting Dose	Maintenance Range	Target Level
Testosterone cypionate (SubQ)	50–70mg weekly	50–100mg weekly	Total T 600–1000 ng/dL at trough
Testosterone cypionate (IM)	100mg q2 weeks	100–200mg q1–2 weeks	Total T 600–1000 ng/dL mid-cycle
Topical gel (1.62%)	40.5mg daily (2 pumps)	40.5–81mg daily	Total T 400–700 ng/dL
Pellet	800–1200mg per insertion	Individualized	Total T 700–1000 ng/dL at 4–6 wk draw
Note	Always draw morning fasting labs	—	Free T often more important than total T. Check SHBG

MONITORING SCHEDULE – TRT

Timepoint	Men	Women
Baseline	Total T, free T, E2, LH, FSH, PSA, CBC, CMP, hematocrit	Total T, free T, E2, SHBG, CBC, CMP. Mammogram current
4–6 weeks	Total T, free T, E2, hematocrit	Total T, free T, E2. Symptom assessment
3 months	Full panel + PSA	Full panel. Adjust dose based on labs + symptoms
6 months	Full panel + PSA	Full panel. Mammogram annually
Annually	Full panel + PSA + DRE consideration	Full panel + mammogram + Pap per guidelines

MANAGING SIDE EFFECTS BY GENDER

MEN – COMMON SIDE EFFECTS & MANAGEMENT

High E2 (>40 pg/mL): anastrozole 0.5mg 2x/wk or reduce T dose

High hematocrit (>52%): therapeutic phlebotomy, reduce dose, increase hydration

Testicular atrophy: HCG 500IU 2x/wk preserves testicular function and fertility

Acne: benzoyl peroxide wash, dietary changes. Rarely dose adjustment

Mood changes early in treatment: usually normalizes. If persistent – check E2 and levels

Hair thinning: genetic predisposition. Finasteride if appropriate after discussion

WOMEN – COMMON SIDE EFFECTS & MANAGEMENT

Acne: reduce dose. Benzoyl peroxide. Spironolactone rarely needed if using appropriate T doses

Facial hair growth: reduce dose. This is most common reason women stop TRT

Voice deepening: PERMANENT at high doses – start low, monitor closely. Irreversible

Clitoral growth: dose-dependent. Reduce if bothersome – partially reversible

Hair thinning (androgenic alopecia): reduce dose. Rogaine if needed

Irritability / mood changes: often resolves. Check levels – may be too high or too low